3-3 Can't intubate, can't oxygenate (CICO) v.1

This is the last resort when all other attempts to oxygenate have failed.

START

❶ Check optimal airway management is in place and maintain anaesthesia: supply 100% oxygen either by tightly fitting facemask, supraglottic airway device or nasal high flow.

❷ Consider ONE final attempt at rescue oxygenation via upper airway if not already done.

❸ Declare CICO and call for help (additional staff and surgical airway expertise e.g. ENT, ICU).

❹ Call for airway rescue trolley and then cardiac arrest trolley.

❺ Give neuromuscular blocking drug if not done so already.

❻ Open CICO kit on airway trolley (see Box B).

❼ Check that the patient is positioned with full neck extension.

❽ Operator position:

- **Right-handed** operator stands on patient's **left** hand side.
- **Left-handed** operator stands on patient's **right** hand side.

❾ Perform a 'laryngeal handshake' to identify the laryngeal anatomy**.**

❿ Perform FoNA using technique in Box C to intubate trachea via cricothyroid membrane. ***(If cricothyroid membrane cannot be identified, use technique in Box D).***

⓫ Secure tube, continue to oxygenate patient and ensure adequate depth of anaesthesia.

BOX A: CRITICAL CHANGES

Cardiac arrest → **2-1**

BOX B: EQUIPMENT INSTRUCTIONS

Airway trolley, Draw D:

- Scalpel with number 10 blade
- Blue Frova Bougie
- Tracheal tube, cuffed, 6 mm

BOX C: (STAB, TWIST, BOUGIE, TUBE TECHNIQUE)

- Identify the cricothyroid membrane ***(If unable, go to Box D)***
- Single transverse incision through skin and membrane
- Rotate scalpel 90^0 with sharp edge facing caudally
- Slide angled tip of bougie past the scalpel into the trachea
- Attach Rapifit connector and circuit with capnography
- Comfirm position of bougie with capnography and reoxygenate
- Remove Rapifit and Railroad tube over bougie

BOX D: IF BOX C FAILS (SCALPEL, FINGER, BOUGIE TECHNIQUE)

- Make an 8-10 cm vertical incision head to toe orientation
- Use blunt dissection to retract tissue to identify trachea
- Stabilise the trachea and proceed as in Box C through the cricothyroid membrane

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